

February 23, 2026

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The Honorable Thomas Keane, M.D., M.B.A.
Assistant Secretary for Technology Policy
National Coordinator for Health Information Technology
U.S. Department of Health and Human Services
330 C Street, SW
Washington, DC 20201

Re: Request for Information—Accelerating the Adoption and Use of Artificial Intelligence as Part of Health and Human Services Programs

Dear Assistant Secretary Keane:

Transcarent appreciates the opportunity to submit comments in response to the Request for Information (RFI) titled *Accelerating the Adoption and Use of Artificial Intelligence as Part of Health and Human Services Programs*, as published in the Federal Register on December 23, 2025.

Transcarent is the One Place for Health and Care™, bringing medical, pharmacy, and point solutions together with the WayFinding™ experience, the first generative and agentic AI-powered health and care platform for health consumers. Our WayFinding experience, paired with transparent pharmacy care, 2nd.MD expert medical opinions, and virtual primary care, integrates with our specialty care for cancer, surgery, and weight health to support people with all their health needs, simple or serious. More than 1,700 employers and health plans and 21 million Members rely on us to provide information, guidance, and care, empowering health consumers with more choice, an experience they love, access to higher-quality care, and lower costs.

How Transcarent Uses AI

Our experience deploying artificial intelligence, including generative and agentic AI, within HIPAA-compliant environments to support clinical care, navigation, and administrative simplification informs the perspectives below.

Transcarent's AI strategy is built to enhance, not replace, the relationship between people and their care teams. Our AI tools serve as a force multiplier for clinicians and a trusted companion for individuals navigating a complex system.

- **WayFinding:** Launched in May 2024, WayFinding is the first application of generative AI focused on improving the experience of the health consumer. By combining instant access to care providers with benefits navigation and clinical guidance on one easy to use platform, WayFinding personalizes each individual's experience, orienting and

guiding them through their health and care journeys, with human support never more than a tap away. New agentic AI capabilities further enable symptom triage, provide AI scheduling for doctor's appointments, and personalized care paths.

- **Clinical Support:** Transcarent integrates AI into our affiliated virtual clinic to enhance efficiency and safety. Before a visit, an AI assistant gathers relevant history and symptoms, structuring inputs for the clinician to review. This reduces administrative burden and allows clinicians to focus on diagnosis and care planning. The AI can also assist in post-visit documentation, follow-up, and care gap reminders.
- **Chronic Condition Prevention and Risk Identification:** AI models identify Members at elevated risk for costly or preventable conditions by analyzing claims, engagement patterns, and onboarding surveys. These insights prompt timely outreach, health literacy, and intervention to avoid unnecessary progression of disease.

Policy Priorities for Responsible AI in Healthcare

While we are optimistic about the potential for AI to enhance care quality, access, and efficiency, we urge the Administration to establish a thoughtful policy framework that ensures safety, equity, and innovation.

1. *Transparency and Explainability:* Patients and clinicians must understand when AI is involved in the decision-making process and have access to clear explanations of AI-generated recommendations.
2. *Privacy and Data Protections:* AI tools handling personal health information must be subject to robust privacy and security safeguards, including modernized federal privacy standards that cover digital health tools not subject to HIPAA.
3. *Bias Auditing and Equity-by-Design:* Developers and deployers of AI systems must test for bias and ensure tools work equitably across diverse populations. AI has the power to reduce structural barriers in health care and should be designed and governed intentionally.
4. *Role of Clinicians and Human Oversight:* AI should augment—not override—clinical judgment. Clinical AI tools should preserve human decision-making and avoid automating care denials or coverage decisions.
5. *Certification and Risk-Based Regulation:* Federal oversight should distinguish between high-risk, clinical AI applications and lower-risk tools such as administrative navigation. Regulatory clarity is needed, but it must be risk-proportionate and not stifle innovation.
6. *Support for Health Care Workforce Enablement:* Federal investment is needed to build AI fluency across the health care workforce, including training for clinicians, administrators, and digital health developers.

We also acknowledge that structural and regulatory barriers limit responsible and scalable adoption. We offer the following recommendations to help HHS accelerate safe and effective AI integration across its programs.

1. What are the biggest barriers to private sector innovation in AI for health care and its adoption and use in clinical care?

From Transcarent's perspective, the lack of harmonization among state AI and privacy laws—combined with the absence of a coherent national framework governing healthcare AI—creates significant operational, compliance, and innovation challenges. Healthcare organizations operating across multiple states must navigate a patchwork of evolving and, at times, inconsistent requirements related to data use, algorithmic transparency, consumer notice, bias audits, and automated decision-making. This fragmentation increases compliance costs, complicates product design, and can slow the deployment of tools that improve access, reduce administrative burden, and enhance care coordination. In the healthcare context, where AI systems often rely on multi-state data flows, employer-sponsored coverage spanning jurisdictions, and interoperability across provider networks, a lack of national alignment risks creating uncertainty for providers, patients, and developers alike. A consistent federal framework that harmonizes privacy protections, governance expectations, and oversight standards would better support responsible innovation, strengthen patient trust, and ensure equitable access to AI-enabled health solutions across the country.

Another barrier is workflow integration. AI tools layered onto existing clinical or administrative processes without thoughtful redesign risk increasing burden rather than reducing it. Adoption is strongest when AI reduces documentation demands, synthesizes relevant patient information prior to encounters, and simplifies navigation for patients and caregivers.

Misaligned incentives further constrain investment. When payment systems reward volume rather than outcomes, providers and organizations have limited financial rationale to deploy AI tools that reduce unnecessary utilization or administrative waste. In contrast, in value-based arrangements, AI-enabled navigation and care coordination can measurably reduce inappropriate site-of-care utilization and avoid unnecessary procedures while improving patient engagement.

Clinicians and patients require clarity regarding oversight, monitoring, explainability, and responsibility when AI influences clinical workflows.

2. What regulatory, payment policy, or programmatic design changes should HHS prioritize to incentivize the effective use of AI in clinical care and why? What HHS regulations, policies, or programs could be revisited to augment your ability to develop or use AI in clinical care?

Transcarent supports the development of a risk-based, technology-neutral federal framework for AI, that applies to healthcare, that promotes safety and accountability without unnecessarily constraining innovation. Federal policy should clearly distinguish between high-risk clinical decision support tools that directly inform diagnosis or treatment and lower-risk administrative or navigation applications that streamline scheduling, benefits guidance, documentation, or care coordination. Overly broad or undifferentiated regulation could inadvertently slow the deployment of AI tools that reduce administrative burden and improve access without materially increasing clinical risk. A proportionate approach—grounded in the function and risk profile of the application rather than the underlying technology—will better protect patients while enabling responsible innovation and scalable adoption across healthcare settings.

HHS should also provide practical, implementation-oriented guidance under the HIPAA Privacy Rule clarifying permissible data uses in AI development and deployment, expectations for de-identification, safeguards against re-identification risk, and audit requirements. Clear guidance will reduce compliance uncertainty while preserving patient trust.

With respect to payment policy, HHS should prioritize outcome-based program designs rather than establishing payment solely for software artifacts. Where Medicare payment frameworks apply, policies should recognize AI-enabled services when they demonstrably improve access, quality, equity, or affordability. Incorporating AI-enabled capabilities into value-based care models and quality programs will more effectively align incentives than stand-alone reimbursement for technology.

3. For non-medical devices, we understand that use of AI in clinical care may raise novel legal and implementation issues that challenge existing governance and accountability structures (e.g., relating to liability, indemnification, privacy, and security). What novel legal and implementation issues exist and what role, if any, should HHS play to help address them?

The integration of AI into clinical environments raises important questions regarding accountability and oversight. These include allocation of responsibility among developers, implementers, clinicians, and organizations; governance of model updates and performance drift; documentation of human oversight; and maintenance of auditable records.

HHS can support responsible innovation by issuing practical guidance on governance expectations for AI used in clinical decision support and adjacent use cases. Such guidance should emphasize transparency, monitoring, incident response protocols, and clear communication when AI tools are used in patient-facing interactions.

4. For non-medical devices, what are the most promising AI evaluation methods (pre- and post-deployment), metrics, robustness testing, and other workflow and human-centered evaluation methods for clinical care? Should HHS further support these

processes? If so, which mechanisms would be most impactful (e.g., contracts, grants, cooperative agreements, and/or prize competitions)?

Robust evaluation is essential to maintaining trust and ensuring safety. Pre-deployment validation should include testing on representative real-world populations and workflows, assessment of subgroup performance and calibration, robustness testing for data shift, and human factors evaluation.

Post-deployment, continuous monitoring should assess performance drift, workflow impacts, safety signals, administrative burden reduction, and total cost of care outcomes. The distinction between technical model performance and real-world impact is critical; successful integration into operational environments often determines measurable outcomes.

HHS can accelerate progress by supporting pragmatic, multi-site research initiatives that evaluate clinical outcomes, equity impacts, and administrative burden reduction in live operational settings. Shared benchmarks and validation infrastructure would facilitate the scaling of responsible AI solutions across programs.

5. How can HHS best support private sector activities (e.g., accreditation, certification, industry-driven testing, and credentialing) to promote innovative and effective AI use in clinical care?

To support private-sector testing and credentialing, HHS should harmonize expectations across federal programs and encourage standardized reporting, auditability, and transparency. Recognition of credible third-party evaluation frameworks would allow scalable, consistent assurance across settings.

Transcarent's AI-powered solutions are built for healthcare from the ground up. Transcarent acts as a HIPAA-covered entity and is certified to HITRUST, SOC 2, and ISO 27001. Our governance model is guided by the Organization of Economic Co-operation and Development (OECD), the first intergovernmental standard on AI that promotes innovative and trustworthy AI, and the National Institute of Standards and Technology (NIST) whose risk management framework incorporates trustworthiness considerations in the design, development, deployment, and use of AI systems. We apply these principles through human-in-the-loop testing, code review, and regular audits and monitoring to prevent bias, protect privacy, and ensure safe, ethical use. The company's generative AI products are developed under clinician oversight. These capabilities are built to support consumers in navigating their care, not to diagnose, treat, or replace clinical judgment.

Further, as a member of the [Data & Trusted AI Alliance](#) (D&TAA), Transcarent collaborates with leading global companies to create transparent and trustworthy AI data standards that promote responsible use and provide real value to inform policy development. Together, we share a conviction that the future of business will be powered by the responsible use of data and AI, including the first of its kind cross-industry data provenance standards. We believe

that trust in data requires understanding its origin—where it comes from, how it was created, and whether it may be used legally. Transparency is critical.

6. Where have AI tools deployed in clinical care met or exceeded performance and cost expectations and where have they fallen short? What kinds of novel AI tools would have the greatest potential to improve health care outcomes, give new insights on quality, and help reduce costs?

At Transcarent, we believe AI holds great promise to revolutionize care delivery for the better – to speed access to treatment, reduce administrative burdens on clinicians, and democratize access to healthcare resources. For example, a person’s zip code no longer matters when they want to meet with an endocrinologist. We also believe that AI should be used only for the right reasons – it should not be used to deny access to appropriate, medically necessary care or in a way that diminishes the quality.

In healthcare, some tasks require deep clinical expertise, while others are necessary but administrative and labor-intensive. Generative AI is already demonstrating strong capability in the latter, particularly in information synthesis, documentation support, and form completion. Given that administrative activities account for a significant share of healthcare spending, these applications can meaningfully reduce burden and clinician burnout. Beyond administrative efficiency, the greater opportunity lies in AI-enabled triage, navigation, and personalization. The healthcare system is complex and difficult for many individuals to navigate. Generative AI can simplify access by guiding patients through coverage questions, scheduling, and care options, helping them focus on their health rather than system logistics.

AI also offers an opportunity to move beyond “one-size-fits-all” care. Personalized educational materials tailored to a patient’s health literacy, language, and specific questions can improve understanding and engagement. When used alongside clinicians, generative AI can extend conversations, clarify guidance, and support informed decision-making.

Additional promising applications include personalized behavioral health support, chronic disease risk reduction planning, synthesis of medical research for clinicians and families, medical education, and guidance through complex treatment decisions.

7. Which role(s), decision maker(s), or governing bodies within health care organizations have the most influence on the adoption of AI for clinical care? What are the primary administrative hurdles to the adoption of AI in clinical care?

Transcarent maintains a formal AI Governance Committee (AIGC) to oversee the development, deployment, and monitoring of artificial intelligence tools used in Member-facing products and internal operations, including within our affiliated medical groups. The AIGC operates under a human-centered framework grounded in principles of safety, security, privacy, transparency, equity, explainability, respect, and data minimization. Transcarent

designs AI systems to augment—not replace—clinician judgment, and does not use AI to deny medically necessary care or override provider decision-making. All AI-enabled services are deployed within HIPAA-covered and HITRUST-secured environments, with ongoing monitoring, bias mitigation efforts, and clearly defined human oversight to ensure accountability, patient trust, and alignment with high-quality, affordable care.

The AIGC includes representation from leaders across internal teams including technology, security, privacy, legal, human resources, clinical care, and marketing.

8. Where would enhanced interoperability widen market opportunities, fuel research, and accelerate the development of AI for clinical care? Please consider specific data types, data standards, and benchmarking tools.

A significant barrier to AI adoption in clinical and operational settings remains fragmented and inconsistent access to high-quality clinical and administrative data. Variability in implementation of interoperability standards, inconsistent semantic normalization across systems, and limited access to longitudinal claims data impede model development, validation, and real-world deployment. As we have previously emphasized in comments to HHS and Congress, access to comprehensive claims and cost data is essential to enable value-based arrangements, measure performance, and support longitudinal care optimization.

Even where interoperability frameworks exist, inconsistent timeliness and completeness of information reduce reliability. AI systems embedded in care delivery require accurate, current, and comprehensive data to function safely and effectively.

9. What challenges within health care do patients and caregivers wish to see addressed by the adoption and use of AI in clinical care? Equally, what concerns do patients and caregivers have related to the adoption and use of AI in clinical care?

Patients and caregivers prioritize timely access to appropriate care, simplified navigation, transparent pricing, and understandable information tailored to their needs. At the same time, they express concerns regarding privacy, bias, transparency, and accountability.

Strong safeguards under HIPAA, clear notification when AI is used in care processes, and accessible human support are essential to maintaining trust. Our experience demonstrates that AI systems designed to augment—not replace—clinical judgment and human advocacy are more likely to achieve sustained engagement and measurable improvements in experience and outcomes.

10. Are there specific areas of AI research that HHS should prioritize to accelerate the adoption of AI as part of clinical care?

a. Are there published findings about the impact of adopted AI tools and their use clinical care?

b. How does the literature approach the costs, benefits, and transfers of using AI as part of clinical care?

In our experience, AI has met or exceeded expectations when applied to reduce administrative burden, improve care navigation and triage, personalize educational materials, and help individuals efficiently access appropriate in-network care. AI-enabled navigation and virtual care integration can reduce inappropriate site-of-care utilization and improve engagement, particularly when paired with human support.

When AI deconstructs the complexities of documentation, approvals, and benefit decision trees to guide individuals through their healthcare journey, people can use all their benefits more easily and efficiently, understand why and what is happening with their health at any given time, and feel confident that they are maintaining costs while achieving the best outcome. When we empower people with the tools to find their way to better health, everyone benefits- Members, providers, payers, and employers.

Conversely, AI has fallen short when deployed without reliable access to complete data, when layered onto existing workflows without redesign, or when governance and monitoring processes are insufficient. Sustainable success requires both technological capability and structural alignment.

Conclusion

Artificial intelligence is already improving access, reducing administrative burden, enhancing personalization, and supporting value-based care across HHS programs, but there is immense potential to do much more. Realizing this potential requires continued federal leadership to advance interoperability, clarify privacy expectations, align incentives with measurable outcomes, and promote consistent governance standards.

Transcarent appreciates the opportunity to provide these comments and stands ready to work with ASTP/ONC, CMS, and other HHS agencies to ensure AI-enabled solutions improve quality, affordability, equity, and patient trust.

Sincerely,



Leslie Krigstein
Vice President, Communications & Government Affairs
Transcarent